

BIOSIMILAR READINESS • ASSESSMENT

The Biosimilar Opportunity Your Plan Is Missing.

Six factors for whether your plan is capturing biosimilar savings, or leaving them on the table because the PBM's formulary serves rebate revenue over your bottom line.

01 Before You Score: Data to Pull from Your PBM

Request these by therapeutic category for every drug class with an approved biosimilar. The third (net cost comparison) is the single most diagnostic request. PBM resistance to providing it is itself a finding.

- ☐ Current formulary placement for each biosimilar vs. reference brand
- ☐ Member cost-sharing tier for biosimilar vs. reference brand
- ☐ Net cost comparison (after rebates) for each therapeutic category
- ☐ Biosimilar dispensing rate by drug, last 12 months
- ☐ Plan document language addressing biosimilar substitution
- ☐ Provider outreach activity from PBM on biosimilar adoption

02 Score Your Readiness

FACTOR	STRONG	MODERATE	WEAK	SCORE
Formulary positioning	STRONG Preferred tier; lower cost-share than brand.	MODERATE Same tier as brand; no incentive.	WEAK Higher tier than brand, or off- formulary.	
Auto-substitution policy	STRONG Plan document addresses biosimilar substitution per state law.	MODERATE State law allows; plan document silent.	WEAK No plan language; substitution unclear.	
Rebate transparency	STRONG Net cost comparison provided per category.	MODERATE Aggregate rebate reporting only; no net cost.	WEAK No net cost comparison; rebate detail opaque.	
Provider education	STRONG Active PBM prescriber outreach; clinical summaries.	MODERATE Outreach offered but not consistently activated.	WEAK No provider education; awareness left to chance.	
Member communication	STRONG Member comms name biosimilars; incentive explained.	MODERATE General member comms; biosimilars surfaced reactively.	WEAK Members first hear at the pharmacy counter.	
Contract language	STRONG Biosimilar dispensing guarantees with remediation.	MODERATE Biosimilars referenced; no dispensing or net- cost guarantees.	WEAK Silent on biosimilars; PBM has no obligation.	

03 Optimization Actions by Factor

IF WEAK ON FACTOR 1 · FORMULARY POSITIONING

Move biosimilars to preferred tier with lower cost-sharing.

Tier placement is the most direct lever. Request a formulary update for every category where a biosimilar is available, with the biosimilar at preferred status and member cost-sharing materially below the brand. Expect rebate-related pushback; the net cost comparison from Factor 3 is your counter-argument.

IF WEAK ON FACTOR 3 · REBATE TRANSPARENCY

Request the net cost comparison this quarter.

For every category with a biosimilar, request a written net-cost-after-all-rebates comparison: brand vs. biosimilar. If the PBM resists or only offers aggregate rebate metrics, escalate. The single data request that changes the entire conversation.

IF WEAK ON FACTORS 4 OR 5 · EDUCATION / COMMUNICATION

Activate provider and member outreach before the next renewal.

Most PBMs have provider outreach programs and member communication templates available; activation is on the plan sponsor to request. Build a communication plan with prescriber-targeted clinical evidence and member-targeted cost-sharing benefit messaging. Time it to formulary changes.

IF WEAK ON FACTOR 6 · CONTRACT LANGUAGE

Add biosimilar-specific provisions at next renewal.

Biosimilar dispensing-rate guarantees, brand-to-biosimilar transition timelines, net cost optimization requirements, and remediation if adoption targets miss. Of the hundreds of contracts we review annually, biosimilar-specific language remains uncommon. The renewal redline is where the gap closes.